A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

17 April 2026

Dr Michelle Wellington
Turn The Corner Clinic

Dear Dr Wellington,

Re: Miss Keely Smith
DOB: 1987-12-04
Address: Unit 83 / 26-36 High Street, NORTHCOTE, VIC, 3070

Summary

- **Normal gastroscopy and colonoscopy with no evidence of coeliac disease, Helicobacter pylori, or inflammatory bowel disease.**
- **Persistent elevation of faecal calprotectin requires further investigation via repeat stool testing and intestinal ultrasound.**
- **Iron deficiency likely secondary to menstrual loss, blood donation, and dietary factors.**
- **Surveillance colonoscopy recommended in five years due to family history.**
- **No action required by you.**

I had the pleasure of conducting a review with Keely Smith following her gastroscopy and colonoscopy. These investigations were prompted by a positive faecal occult blood test, an elevated faecal calprotectin level, and a significant family history of malignancy.

I am pleased to report that the investigations were essentially normal. Gastric biopsies were negative for *Helicobacter pylori*, and duodenal biopsies excluded coeliac disease. The colonoscopy revealed no macroscopic or histological evidence of ulcerative colitis, Crohn's disease, or microscopic colitis.

We discussed her symptoms in detail. While initially reported as asymptomatic, Keely noted episodes of loose stools coinciding with ovulation. Regarding her iron deficiency, she continues to have periods and is a regular blood donor. Given the negative endoscopic findings, her low iron stores are likely attributable to the combination of physiological loss and dietary intake rather than

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

an occult gastrointestinal source. I have advised that a small bowel capsule endoscopy is not indicated at this time.

Impression and Plan To further investigate the elevated faecal calprotectin and the remote possibility of isolated small bowel Crohn's disease, I have requested a repeat calprotectin stool test to determine if the previous result was an anomaly. I have also referred Keely to St Vincent's Hospital for an intestinal ultrasound to evaluate the small intestine.

In light of her family history, specifically a grandmother diagnosed with colorectal cancer in her 60s and a sister with endometrial cancer, we discussed future surveillance. The sister's history of endometrial cancer raises the possibility of a genetic link to colorectal malignancy. I have suggested a conservative surveillance interval of five years for her next colonoscopy. Alternatively, it would be acceptable to commence the National Bowel Cancer Screening Programme at age 45.

I will review the results of her upcoming tests and will notify Keely and yourself should any further action be required. If the results are normal, no further formal review is scheduled. Thank you very much for this referral.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

SMITH, KEELY JANE : SVP 924617

DOB: 04/12/1987 Age: 38 years

Unit 83, 26-36 High St, Northcote 3070

Results from 27/03/2026 14:50 Episode# 26P213348 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

Lab #: 2026B008155 **CLINICAL NOTES** Iron deficiency. Increased Calpro. Gas = gastritis. Duo = N. TI = N. Random colon = N. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric bx". A mucosal fragment measuring 5 mm. A1. 2. Specimen labelled "Duodenal bx". Three mucosal fragments measuring 1-4 mm. A1. 3. Specimen labelled "TI bx". Two mucosal fragments measuring 2-3 mm. A1. 4. Specimen labelled "Random Colon bx". Four mucosal fragments measuring 1-3 mm. A1. **MICROSCOPIC DESCRIPTION** 1. The sections show specialised and non-specialised gastric mucosa with normal architecture. Small numbers of chronic inflammatory cells are present in the lamina propria, but no acute inflammation is seen. No organisms are identified, and there is no atrophy, intestinal metaplasia, or dysplasia. There is some smooth muscle between glands. 2. The sections show duodenal mucosa with normal architecture, and in particular no villous atrophy. Normal numbers of chronic inflammatory cells are present. No organisms are seen. There is no atypia. 3. The sections show small intestinal mucosa with normal architecture and a normal complement of inflammatory cells. No organisms are seen. There is no atypia. 4. The sections are of colonic type mucosa with normal architecture. There is no increase in inflammation and no organisms are identified. There is no evidence of microscopic colitis. There is no dysplasia. **PATHOLOGIST'S OPINION** **1. Gastric: Mild inactive chronic gastritis. 2 - 4. Duodenum, ileum, colon: Within normal limits.** Pathologist: Dr Daryl Johnson

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065